Journal of Substance Abuse Treatment 131 (2021) 108564

Contents lists available at ScienceDirect

Journal of Substance Abuse Treatment

journal homepage: www.elsevier.com/locate/jsat

The effects of drug addiction treatment methods on families' behaviors: The Congress 60 treatment method

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ARTICLE INFO

Keywords:
Addiction treatment
Family
Grounded theory

ABSTRACT

Background: When a family member who uses drugs begins drug abstinence, this change can cause a great deal of tension within a family. Although family members are willing to help drug users overcome their addiction, they sometimes disrupt the treatment process and, in many cases, resist change altogether.

Method: This qualitative research study uses grounded theory to study the factors that contribute to positive family change and changes in the family system throughout the treatment process. The study selected 39 participants from among the clients at Congress 60 Human Revivification Society using purposive sampling until the study reached data saturation. The study collected information from participants during semistructured interviews. The participants consisted of 18 men who used drugs and 21 individuals from their families who had undergone one year of treatment at Congress 60.

Results & discussion: The findings of the current study show that peace in the family is an influential factor that helps drug users overcome their addiction. Clear boundaries, detachment, and reduced emotional distance were the major structural changes that participating families exhibited after educational interventions, positive family support, and they gained trust in the treatment method.

1. Introduction

Family strongly influences people who use drugs (PWUD) and shapes PWUD's behavior (Minuchin, 2018). Based on a report from the Iran Drug Control Headquarters in 2015, 2,808,000 people between 15 and 64 years old were dependent on illegal drugs (Ghane et al., 2018). If each family has four members, about 11.2 million individuals are affected by addiction and its consequences, such as high stress, health problems (Copello et al., 2009; Ray et al., 2009), family conflicts, and financial difficulties (Orford et al., 2013; Weisner et al., 2010). Evidence confirms that one's family impacts their addiction entry, treatment, and retention (Kourgiantakis & Ashcroft, 2018), and so family members are an integral part of treatment.

Substantial evidence indicates that family involvement can positively expedite treatment entry and treatment course completion while improving treatment results (Kourgiantakis & Ashcroft, 2018). However, little research exists on family changes and the factors contributing to such changes. Current studies on this topic indicate that empathy and positive support are associated with greater retention and reduced behavioral problems among PWUD (Knight & Simpson, 1996; Noroozi et al., 2018). Despite the significance of the family atmosphere in addiction treatment, PWUD often find it difficult to obtain positive support from dysfunctional or deviant families (Al Ubaidi, 2017).

Individual and family growth are interlinked—thus, as one of the system's members grows, other members must either adapt themselves or pressure the transforming member to revert to their previous state (Minuchin, 1979). Hence, if family members fail to keep pace with the member undergoing treatment, they will inevitably prolong the PWUD's addiction. In other words, addiction supports the inner balance of the family (Lettieri et al., 1980). Faced with a threat or disturbance, the family—similar to any other system—attempts to preserve its vital balance and stability (Romijn et al., 1991). In this regard, Minuchin (1979) claims that family members often unwittingly act in a hostile way in such circumstances because they are chronically trapped in familiar patterns of interaction (Minuchin, 1979). Consequently, the intention to preserve one's vital family balance explains why some families resist change and why individuals continue to use drugs despite their attempts to stop and their families' attempts to help them stop.

One significant problem in dealing with this resistance is the inability of the therapist to change the perspectives of the family (Solomon,

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https://doi.org/10.1016/j.jsat.2021.108564
Received 29 July 2020; Received in revised form 6 June 2021; Accepted 4 July 2021
Available online 15 July 2021
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1974). A family's abilitie to cope with such resistance and influence the structures instilled in the family depend significantly on that family's cultural and social circumstances. Research shows that culture-based interventions can involve the family in the treatment process (Gainsbury, 2017), thereby decreasing their treatment resistance (Jones et al., 2018). Consequently, when treatment methods do not provide such cultural support, therapists cannot incorporate families into the treatment process effectively.

Along with the importance of cultural sensitivity of treatment methods, intervening in the complex communication among family members and moving them toward a change entail mutual understanding, flexibility, and adjustment from PWUD's families and treatment service providers (Center for Substance Abuse Treatment [CSAT], 2004). To integrate a family into the treatment process and change their dysfunctional behavior patterns, the treatment service provider needs to challenge the family's current behavior patterns without blaming such patterns for the problem (Hadfield, 2000).

In other words, “support” and “exposure” are two crucial elements with which service providers can help families change (Epstein, 2000). In addition to the effective treatment techniques that can change the behavior of PWUD and their family members, treatment conditions (Amram & Benbenishty, 2014) and the characteristics of the health care system (Rapp et al., 2006) also determine whether PWUD engage in the treatment process. Other factors that can attract PWUD and their family members to the health care services (or deter them) include access to treatment (Holt et al., 2007; O'Brien et al., 2019), treatment costs (Holt et al., 2007; Owens et al., 2018), time conflicts (Rapp et al., 2006; Xu et al., 2008), misinformation about the treatment experience (Matusow et al., 2013), organizational climate (Lundgren et al., 2012; Lundgren et al., 2013), and the level of trust in treatment systems (Liebling et al., 2016).

Congress 60 Human Revivification Society (C60), an addiction treatment center in Iran, has managed to considerably increase the participation rate of families with PWUD. Knowledge about effective methods for incorporating families into the treatment process and improving specific aspects of these families can be beneficial to therapeutic interventions. The current study aims to identify the mechanisms underlying effective changes in the family system throughout the treatment process at C60 and examines the subsequent changes that occur in families after benefiting from this treatment method.

C60 is a self-help group providing addiction treatment to women and men in Iran. The C60 method includes 10–11 months of gradual dose reduction, opium tincture (OT) assisted pharmacotherapy, and intensive psychosocial interventions. Along with reducing the patient's drug doses, families take part in psychological, social, and recreational activities. C60 stresses the importance of families' presence—in most cases, a PWUD is accepted depending on whether their families will participate. Families can also engage in the treatment process without the PWUD's presence.

C60 remains in touch with most participants and their families after the treatment period (11 months) (Dezhakam, 2013). Training in C60 often occurs in groups. The PWUD and their family members are trained in separate groups (known as legions). However, patients and their families have access to individual counseling sessions, pamphlets, and a package of CDs containing educational audio files that help them to deal with problems that arise throughout the treatment process. A leader provides group training and individual counseling. C60 stresses individual responsibility and shifts the focus away from the environment. This program focuses primarily on Iranians (as most Iranians have inherited cultural characteristics such as fatalism and external attribution [Pilevari & Zahedi Asl, 2015]) and people with addiction who have easy access to drugs (see https://www.congress60.org/EnUs/Congress-60-Human-Revivification-Society-The-Addiction-Treatment, for more).

2. Method

We conducted this research using the grounded theory method. We selected participants from among the PWUD and their families referred to C60 in Tehran in 2017. We conducted the interviews at the Central Branch and Taleghani Park, where people from all branches of C60 gather. We used purposive sampling and continued data collection simultaneously with data analysis until the theoretical saturation stage.

We prepared an interview guide before we started interviewing. For this purpose, we conducted 18 interviews with people who were treated the previous year and 21 interviews with individuals from their families. The questions were open-ended, tailored to the participants' understanding and experiences; we designed them based on the general question of the study (e.g., How do you think the family affects addiction treatment?). We gradually focused on specific issues, and, if needed, study staff asked exploratory questions (e.g., Can you explain it? Could you give an example?). The interview guide also focused on PWUD's relationships with their family members during and before treatment, their closeness to and trust in therapists and family members, their sense of responsibility in their mutual relationships, the relationships among other family members, how relatives intervene in the family, and how family members relate to their friends and relatives.

We selected participants from among a group of volunteers who met the following criteria: (1) individuals who had completed the 11-month treatment course at C60, (2) men who had successfully ceased drug use in the last year, (3) individuals who were 18–55 years old, (4) individuals who lived with their families, and (5) individuals who did not take any antipsychotic drugs and were cognitively able to answer our questions. The study applied the following criteria for family members in our study: (1) family members whose PWUD had completed at least 11 months of treatment in C60 and (2) individuals who had received treatment from C60 for at least 18 months. They may have attended C60 before, after, or during the PWUD's treatment.

Study staff transcribed the audio file immediately after each interview. After analyzing each interview, we conducted the subsequent interview. We analyzed all manuscripts using the Corbin and Strauss coding method (Corbin & Strauss, 2014).

Before conducting the interviews, we attended group meetings held at C60 for about a year and a half. Our long-term and ongoing involvement with the environment and the participants gave us a deeper understanding of the data. We used an expert check to ensure the validity of our research. For this purpose, two experts reviewed the data, concepts, and categories to ensure that the categories matched the participants' statements. The two experts were a C60 therapist and an expert in qualitative research methods. After removing participants' demographic information, we provided the specialists with texts of the interviews, along with the extracted categories.

We also shared the findings with three of the research participants for review and approval, and made necessary adjustments based on their opinions (member check). A PWUD and two members of their family (a sister and a brother) who majored in psychology and sociology conducted the review.

2.1. Ethical considerations

Allameh Tabatabai University Social Group Work approved the study protocol. The study carried out all interviews with participants' written consent. All participants also gave us permission to record interviews. Before getting the participants' informed consent, we provided them with information regarding the purpose and process of the research, how the data would be reported, the confidentiality of their information, and the nondisclosure of their identities. Study staff informed the participants that they could discontinue the interview at any time. The study guaranteed participants' anonymity by deleting their identities from the transcripts.

3. Results

The average age of individuals who completed the 11-month treatment was 38 years, and their average age of first drug use was 14 years. Five participants had received no previous treatment. The remaining 18 participants with addiction had already been to drug use treatment clinics or mid-term residential treatment centers or had self-medicated—all of them had ceased drug use. Most participants had not completed high school and had earned less than $73 per month in the six months before the study. Ten were married, and all others were single. In addition to those who had undergone treatment, we intereviewed11 wives, three sisters, four fathers, one brother, and two mothers.

3.1. Core category: peace in the family

In our opinion, "peace in the family" was the most prominent theme. The other categories extracted from this research center on this phenomenon and together form a cohesive whole. In other words, our findings revealed that the most significant change occurring in families after treatment is reduced conflict among family members. This change brings peace, which results from a feeling of successful treatment and strengthens the emotional bonds between the PWUD and their family members.

3.1.1. Having a sense of mastery

Most of the PWUD and their family members regarded addiction as manageable. As one PWUD's father pointed out, "*When we came to C60 and received training, we were pretty sure afterward that our son would come here and be treated*." A PWUD told us, "*Back then, I thought that addiction had no cure. But, here in C60, I was given the belief that addiction actually can be cured… And I was cured*." A father noted, "*The treatment of addiction with C60, the opium, and the DST method are all effective. This method is without relapse. No doubt about it*." One PWUD described, "*You will never find any place in the world like C60.* [*PWUD*] *only can be treated in C60 for sure. Not anywhere else*." As a result, family members were better able to control and respond to recurring challenges and situations. In these situations, families attributed the cause of the relapse to issues such as God's permission or the Lord's will. One father noted that "*permission must be issued by God. Be sure; if my other son has relapsed, it's because God's consent has not yet been given. But I'm sure he'll be treated too*." Also, family members acknowledged that they had entered the treatment course primarily to recover themselves rather than to help the PWUD. They believed that it was better to focus on changing themselves rather than changing others. As the brother of one of the PWUD said, "*If I attend the Congress, it is for my own sake. I come to improve myself, to correct my worldview so that I can change my behavior for the better; my brother can follow my example. My behavior can influence him as well so that he develops an interest in the treatment process*." These beliefs made them feel, to a large extent, that they had control over the circumstances.

3.1.2. Emotional development

Families who participated in the study conceded that they had developed more positive feelings toward their PWUD relative. The wife of one of the PWUD pointed out that "*After entering treatment when my husband came home with his drug* (*opium*), *I helped him prepare it. We always had sweets available at home. He would take his drug, and I would bring him tea with sweets, telling him*, "*Enjoy your tea*!'"" A mother said, "*I prepared him his drink* (*opium*) *and took it to him in a saucer*. '*Thanks*,' *he would say*. '*Enjoy*,' *I would reply*." Families no longer dealt with the PWUD by getting angry and having arguments. Instead, they gave the PUWD attention and showed concern. As a result, family members found new opportunities to communicate differently. The wife of one of the PWUD noted the following: "*I used to wait a while, and I wouldn't say anything to my husband. But, after three months, I exploded. I was bothering both my husband and myself. I learned in Congress that I should not expect my husband to change in such a short time. I should have given more time to him and myself*."

Moreover, PWUD's perceptions of the negative behavior of their family members had also changed. One PWUD said, "*I used crystal and opium at the same time. My behavior had severely deteriorated. Now, I am just realizing and wondering that how I was treating my family back then. I always complained about their behavior. Now, I understand that they were such a great family who did not kick me out of the house*." Another PWUD said, "*When you see your darling is destroying themselves with drugs, you will be destroyed too*! *You try everything possible to help them, but since you don't have the necessary knowledge and skills, you do everything wrong*! *And then, the situation only gets worse*." Changes in participants' beliefs resulted in positive emotions; these changes were possible because the PWUD internalized their therapists' instructions.

3.2. Causal conditions

Clear boundaries, detachment, and reduced emotional distance were the main structural changes that we observed in the participating families.

3.2.1. Clear boundaries

Participating families indicated that they could adjust the relationships among family members and the relationship between family members with others (friends, relatives, etc.). Although family members had no rigid communication boundaries with those outside the family, they avoided others' ineffective and sometimes risky interventions in the PWUD's affairs. For example, the sister of a PWUD mentioned, "*Our relationship with relatives became less close than it was ten years ago. When my brother was on drugs, their interference was very high. Now, we no longer let them intervene. Now, they know that they should not give him advice and not interfere with specific issues. We keep our privacy. We do not disrespect anyone. We just protect our privacy*." This balance was also evident in family members' relationships with one another.

3.2.2. Detachment

Family members who had been very close and involved with the PWUD before treatment began to fulfill their own forgotten needs during the treatment. The wife of one of the PWUD pointed out, "*Now, I think more about myself. I pay more attention to myself, such as what I wear. In the past, I even didn't like to change my clothes. I wear makeup now. I was busier with my children. I had cut off contact with everyone. I just wanted to sleep at night. Now, I always wait for my husband to get back and play with the kids together*." One of the mothers participating in the study stated, "*I was in terrible condition, so I went to buy the materials for making artificial flowers. I started to make flowers. My guide had told me to do whatever I could do. I remember when my son opened the door and saw that his mother started making artificial flowers after so many years, he gave me a surprised look*." Engaging in recreational, artistic activities—such as knitting, embroidery, carpet weaving, playing sports, spending time with friends and family, joining a library, and keeping a pet—reduced family members' rumination and the stresses that come with it.

3.2.3. Reduction of emotional distance

The experiences of the PWUD and their family members in this study indicate that changes in family patterns are possible and that the treatment at C60 can reduce emotional distance among family members. Often one member of the family assumes responsibility for the PUWD, while other family members generally lack involvement. As one father noted, "*My wife used to take my son to school, bring him back home, and help him with his homework. She did the same thing during his drug abuse. She has completely forgotten me*! *I thought she was spoiling him with her excessive love. Since we came to C60, she has become less engaged in my son's affairs*."

The establishment of rigid boundaries between involved family members and PWUD, combined with opportunities for interaction between emotionally distant members and PWUD (such as participating in

recreational, artistic activities), changed this pattern. As a result, distant family members and PWUD grew closer. One father said, "*My son was very close to his mother. Now he has become closer to me. His mother would take his side when I was quarreling with him at the time.*" He added that his wife always criticized him for not taking care of their child. He considered this as a barrier to effective communication with his son. He also said, "*My wife thought I didn't take care of my son, but I couldn't get too close to my son.*" This study found that "distant" fathers became closer to their PWUD relative during treatment.

3.3. Intervening conditions

3.3.1. Training

The PWUD and their family members expressed that proper training had significantly affected their mutual understanding of the PWUDs' conditions during drug use, as well as during (and even after) treatment. One of the participating spouses said, "*One of my guide's statements was, 'When your spouse is taking drugs, don't do anything, and don't bother him, and just leave it to your spouse's guide.' With the training I received, I realized that everyone had to do what he or she was supposed to do.*" Another wife said, "*Before attending the C60, every time my husband quit drugs, I faced numerous problems. My sexual relationship used to get more awful. Even when I explained it to my friends, they couldn't believe it, saying that was impossible. But it was in C60 meetings that I found out sexual problems may linger for a while after quitting.*" In addition to families, PWUD also stated the importance of proper training in their relationships with others. One PWUD said, "*My wife used to shout at me sometimes, saying she wanted to get a divorce. Sometimes she ignored me. She was also untrained. She did not know how to deal with me and did not let me get out of the house. We were always fighting. The fights used to help me make excuses and ditch the house.*"

3.3.2. Spouses' family-of-origin support

When the wives of PWUD had supportive families and knew that their families would support their divorcing their husband, they had less difficulty interacting with their husband while they were still using drugs. As one of the participating spouses noted, "*My dad wasn't willing to talk to my husband. Every time I went to him, he would just say one sentence, 'Get a divorce anytime; your daddy will support you.' I was the only daughter. My father always encouraged me and said, 'Get a divorce. He is nothing like you. You can't work it out. Everything aside, he doesn't have any money.' I wouldn't listen to my father's words, but I knew my father would support me if needed.*"

However, some factors—for example, age and having children—significantly impacted this support for and from family members. For example, since divorce is culturally, socially, and economically more troublesome for women with children than those without children, women with children put pressure on their husbands to get treated. Consequently, their home environments were tenser. One wife said, "*I filed for divorce, hoping that my husband would leave me. Everyone told me to cancel the lawsuit for the sake of my children. They always tried to dissuade me, and it always worked. I did it three or four times when my husband was using drugs. Life was bitter for all of us back then.*"

Older women often chose to provide a favorable atmosphere for treatment. As one wife told us, "*I never allowed my family to interfere in our affairs. I did not want any of them to stand against my husband and shatter his pride in front of our kid. I always respected him. I did not let anyone find out at all.*"

3.3.3. Positive support

Many PWUD—especially those who were single—tended to use drugs outside the home. Some of the families of single PUWD in our study had given the PWUD permission to use a separate room in their house for their drug use to give them privacy and security. These families left the PWUD alone while they were using drugs. One PWUD's father said, "*Before we came to Congress, my son used to take drugs outside the home, or in his friend's parking lot, or inside the car. But after the training we received from C60, we indirectly told him not to go out. I would say, 'My son, stay home in your room. You'll be less hurt this way' Little by little, he started to take drugs in his room.*"

Granting the PUWD permission to use drugs at home enabled family members to utilize the training they received and encourage the PUWD to get treated. At the same time, PWUD experienced less anxiety by having a safe place to use drugs. Most family members would then indirectly invite the PWUD to engage in treatment. Indirect suggestions were useful, especially among families in which the PWUD had a great deal of power, perhaps because the PWUD did not consider such requests a threat to their free will. One PWUD's sister said, "*In the beginning, C60 gave us a series of training packages. We put those packages on the computer desk. Without asking Hadi to read those packages, we simply put them over there and tried not to bother him at all. We just left it in the room where he visited most. Then we saw him open and look at the packages. Later he said, 'Where are you going?' We said, 'We are going to Congress. Take a look at it and see if you like it.'*"

3.3.4. Trust in the treatment method

Some PWUD and their families believed that service providers in mid-term residential centers suffered from ethical corruption.[1] Thus, they did not feel that women in their families should come to these places to meet the patient or receive training. The wife of one PWUD spoke of her husband's treatment history in a mid-term residential treatment center: "*My husband was opposed to this. He was not at all satisfied with me going to the training classes held in the camp. He said it was not a good place for me.*" *One of the PWUD said,* "*I remember when I was taken to the camp, I did not want my family to meet me there because its environment was not suitable at all.*"

Conversely, C60 made the PWUD and their family members comfortable in the treatment setting. As one father said, "*Here, you and your family are safe among all these PWUD. If it weren't so, the director of C60 wouldn't bring his wife and daughters here.*"

Participants also believed that service providers in C60 aimed to help them and had no intention of economic benefit and financial abuse. One PWUD said: "*Here, the affection between people is genuine. Whenever I call my guide, he answers me. He sits beside us for hours and talks to us. It's unbelievable that he has received no money from us. When we tell others that these people help us without anything in return, they won't believe it. I know in Congress what true love means. What does gratuitous love mean?*" Families had had considerable experience with drug treatment centers that they felt were only after their money. Therapists in such centers had failed to perform the "joining operation" (Minuchin, 2018) with the family. As one PWUD put it, "*I used to attend this center which charged you for every little thing. All they wanted was to make more money.*"

3.4. Contextual conditions

According to participants, the similarity between treatment methods and their own cultural and religious teachings, as well as the participants' trust in the treatment methods, provided a context in which deep-seated patterns in the family could be changed. Such change was accomplished by incorporating family members into the treatment process and decreasing their resistance to the therapeutic interventions.

3.4.1. Similarity of the treatment method to religious teachings

The C60 training method aligns with the teachings of Islam, which has made many individuals more accepting of and familiar with the training. In this treatment method, the therapists did not require the

[1] Mid-term residential treatment centers provide health care for a period of 28 to 90 days. In these centers, a 12-step program is provided to the residents and they are encouraged to participate in self-help groups after being discharged from these centers.

PWUD and their families to engage in self-disclosure. This eased the joining operations process between therapists and the group. As one PWUD said, "*I don't like to talk to others about selling my mom's and my sister's clothes during my addiction. Once members learn about this issue, they will somewhat abuse you. These are the things C60 asks us not to do.*" A PWUD's mother noted, "*My son says that people have to talk about their wife and kids in NA. I don't know how they can do that. I guess they are supposed to talk about the damage they have done to their family at a step. I can never do that, even if they force me to.*"

One of the main principles in C60 is the prohibition of disclosing personal information or stories. In Islam, confession of sin is unnecessary because it eliminates the obscenity of the sin. As Ali (PBUH), the first Imam of Shia Muslims, put it, "Your secret is the reason for your happiness and prosperity only if you hide it, and if you do otherwise, it will ruin your life" (Amedi, 2002). In our culture, we are repeatedly warned to "keep up our appearance" and "be careful about how we appear to others". Therefore, our people are not religiously permitted to disclose personal information to others. If someone is encouraged to disclose personal information as part of a specific treatment, they might make dishonest statements to make themselves and their families look good.

3.4.2. Similarity between the treatment method and features of Iranian culture

Culturally, Iranians often emulate heroes (Pilevari & Zahedi Asl, 2015). PWUD and their families in the current study had made a hero of the C60 director and attributed positive moral characteristics to him. The mother of one PWUD mentioned, "*Before the revolution in Iran, they used to say 'God, King, and Homeland.' If I want to re-write that slogan now, I would say 'God, the director of C60, and homeland' or write, 'God, our salvation angel (the director of C60), homeland.' The director of C60 did not just cure my only child; the primary basis of the teachings he offers come from Qur'anic teachings.*" Another PWUD said, "*I believe in what is being done in C60. Faith emanates from security. C60 gives me security. I'm sure of that. I believe in what the C60 director says, and I'm not biased about it at all. I have been well for six years now. I surrender to the C60 director and the orders carried out in the C60.*"

3.5. Consequences

Participants identified as recovery outcomes gaining a sense of self-worth, removing oneself from isolation, relieving feelings of loneliness, and rebuilding oneself.

3.5.1. Sense of self-worth

The experiences that PWUD and their families shared increased their positive feelings toward one another during treatment. Participants indicated that they felt better about themselves when they ceased using the substance. One father said, "*I used to blame others for my actions, but things have changed now. When you change yourself, so do those around you. Everything depends on you. I changed myself, and then my family changed their attitude towards me*" A mother said, "*I was able to pass that stage, and then I managed to fix my relations. I succeeded in changing myself.*"

PWUD and their families regularly received positive feedback from the people around them. Other families sought counseling from these individuals to treat family members who used drugs. One PWUD said, "*I was the one who provided drugs for the whole neighborhood. Now I am a positive model for the family and the neighbors.*" The approval and encouragement of people around them helped families and PWUD to construct more positive images of themselves.

3.5.2. Eliminating isolation and loneliness

C60 offers opportunities to play various sports, provide service to other patients and their families, and spend leisure time constructively. For instance, many C60 clients gather in Taleghani Park on Fridays. These opportunities have allowed for positive interactions between the recovered individuals and their families. They have also enabled patients to improve their interactions with people outside their families. As one PWUD told us, "*Yesterday was Thursday, and I was off. My wife said she was bored in the house. The kids weren't home either. I said, 'What do you want to do? Where do you like to go? Shall we go to Darabad?' We went there. It was a lot of fun. In the past, I was always thinking of running away from my family. Now I still want to be with them. They were still home because of me and my problems.*"

3.5.3. Rebuilding oneself

Some families were able to give meaning to the experience of addiction and its problems—some even considered it a guide in their lives. They believed that life without addiction could have been worse. Moreover, some families thought that having PWUD in their lives was a blessing from God. Rather than remembering the adverse effects that addiction had had on their lives, they focused on the positive outcomes that the experience had on their lives. "*That I encountered my husband had a reason. I always said that God wanted to test me with this person; otherwise, why did I marry him. God could have put someone else in my way for marriage. Whenever I compared my husband with others, I would say that my husband's single problem is an addiction. Many others have addiction problems, and are at the same time smugglers, have moral problems, and are morally deviant.*"

The PWUD perceived their past experiences as necessary lessons. They also learned specific skills, invited other families and PWUD to seek treatment, competed in athletic competitions, and became more confident that they could successfully continue the treatment process.

3.6. Action strategies

3.6.1. Attending the peer group

This subcategory includes communicating with people with similar problems, joining an extensive network of recovered individuals, and forming new attachments. One of the mothers said, "*The first time I entered Taleghani Park (a meeting place for families and the PWUD at C60), I said, 'Oh my God, there are so many families involved in addiction. I thought we were the only ones who had this problem.' It was reassuring, and I thought there was a glimmer of hope.*" Seeing the crowd of people gathered at Taleghani Park had made her feel less alone. The other PWUD and their families made it easier to endure the problems associated with addiction. Listening to the issues of others made their problems seem smaller. A PWUD said, "*You sit in a group next to someone whose situation is way worse than yours. When you see that, you say 'thank God my situation is so good.'*" A sister of a PWUD told us, "*It took me a few months to come here [at C60] after I found out that my brother was an addict. These few months were so hard for me; each month passed like a year. But here, I have seen people who have been living with a user for more than 15 or 20 years. There is a girl. Her father has been an addict for many years. How are they coping with that?*"

Some group members have been in recovery for several years, while others are still in the early treatment stages. Such heterogeneity among group members made the PWUD more helpful and motivated in their efforts. As one PWUD said, "*When I saw other users in C60, I told myself if they have managed to quit, you can too. You are not old yet. Some of them are 60–75 years old and were treated in C60. Treatment for older adults gave me hope. It gave me the power to move forward.*"

Family members had considerable empathy for each other because of their similar experiences. One of the mothers said, "*When you can easily talk about your problems, you will be able to pour out your emotions; here you can do it; you can talk about things that you cannot bring on anywhere else… to anyone else; you have always been forced to hide these secrets from your family and friends even if they knew it already; But here, you can talk, and everybody understands you.*"

Group training helped family members to develop positive attitudes toward the PWUD. A father said, "*Addiction is a kind of disease. What would I do if I had a child who suffered from MS disease? How would I deal*

with that? Did I not treat them with love and affection? Did I not do everything, even sell whatever I had, to spend it all so that they could recover? Would I mistreat them? Certainly not. Addiction is just a disease." Moreover, new connections obtained through the group reduced families' focus on the behaviors of the PWUD.

3.6.2. Proper design of treatment processes

The dispersal of C60 branches in Tehran had kept individuals and their families of nearly equal socioeconomic status together. A mother noted, "*I was too scared to take my child to camp - mid-term residential centers. They are all at the end of their ropes. There, my son would learn things he does not know, but in C60, he meets people with similar levels of status.*" The wife of another PWUD said, "*During his camp time, my husband always complained about being downgraded since he had to attend classes with homeless people. He always said, 'this is not my place. I have to clean the toilet here.' He did not like that. He was in pain. But C60 is something else. It is different.*" This suitable dispersion and ease of access to C60 held classes and meetings after working hours or on weekends, making it easier for families to attend meetings and group therapy. Another wife said, "*When my husband was in the camp, the camp manager had told him that his family should also attend classes. So, I participated in a class once, but commuting to such a long distance, while carrying a small kid, left me no choice but never to attend the classes again.*"

C60 also fostered the formation of friendship networks. Primary friendship circles played an essential role in attracting other PWUD to the treatment process. One PWUD said, "*I am still in contact with the friends with whom I used drugs in the past. Many of them have come to the C60. After my treatment, I told my friends about this place, this process, and the recovery reality. Here, I was given the belief that addiction has a cure, so I used it, and I recovered. So, I tried to pass on the good deed and help others to quit drugs and get well.*"

C60 also used opium tincture to treat addiction, which helped to eliminate anxiety among PWUD about experiencing withdrawal symptoms. As one PWUD told us, "*I had quit several times by myself. Sometimes, I managed to stay away from drugs for four months, but after that, someone would come to me, use drugs next to me, and make me feel bad. I was craving drugs back then. I was scared of not finding any drugs. Whenever I smelled drugs, I would run toward them. But now, I have changed. I don't care even if a ton of drugs is piled up next to me.*"

In addition to the training that C60 provided, it used proverbs, anecdotes, Quranic verses, and guides' stories from clients' daily lives, presented in simple language and in various forms—for example, individuals participated in groups, read instructional pamphlets, listened to training sessions, and received individual counseling. Accordingly, even families with no or low educational attainment were able to internalize this training.

4. Discussion

The current study revealed that families were more relaxed and conflict between family members was significantly reduced during the C60 treatment process. The PWUD's growing sense of mastery over conditions that had arisen due to their addiction is one of the main factors that created peace and calm within families. The families who participated in this study believed that treatment was available for PWUD that did not involve relapse. They also believed that PWUD can enter the treatment process when the conditions are right. This way, families do not need to become overly concerned when setbacks occur because they know the PUWD will eventually be treated by C60's method.

Families also came to believe that addiction treatment could be achieved when the PWUD decided to change. They also realized that their controlling behaviors did not help the treatment process. They attributed the PWUD's misbehavior to his/her drug needs, which changed the emotions of family members from anger to concern. Such emotions created new opportunities for different types of communication in the family (Rowe, 2012). In other words, after the treatment, PWUD realized that their families' behaviors were a result of their lack of knowledge about how to deal with drug addiction. This realization enabled the PWUD to develop positive perceptions of their family members' attitudes.

Positive emotions contribute to health improvements (Kong & Zhao, 2013; Mukherjee, 2017; Whisman, 2013) and provide a supportive social atmosphere (Gibb, 1964). They also increase acceptance, tolerance, and stability while reducing pessimism, thereby enabling the PWUD to use resistance strategies in risky situations related to drug use (Akhlaghi Yazdi Nejad et al., 2017). On the other hand, negative communication leads to feelings of hopelessness, being misunderstood, dissatisfaction, loneliness, suffering, and mistrust (Anand et al., 2017; Cordova et al., 2013), which serve as powerful motivation for substance use and increased conflict among family members (Anand et al., 2017; Koob, 2013). By creating more rigid boundaries between involved family members and the PWUD, other family members can form close relationships with the PWUD, which was one of the main changes among families that this study observed. In turn, such boundaries can significantly reduce family conflicts and lead to peaceful conditions.

The typical pattern that this study observed among family members was a volatile relationship between the mother or wife and the PWUD, and a lack of involvement from other family members. This pattern is consistent with the results of other studies (Baron et al., 2010; Hoeck & Van Hal, 2012). Disengagement between the PWUD and the emotionally distant family members weakened their ability to communicate and increased the likelihood that the PWUD would engage in behaviors without thinking about others (Ingoglia et al., 2011). However, the intimate relationship that C60 created between the PWUD and the emotionally distant members acted as a motivational-behavioral control system during the treatment process (Parolin & Simonelli, 2016).

This intimate relationship is actualized at C60 through opportunities for the PWUD to interact with disengaged family members. The treatment includes training and sports classes, spending leisure time in places provided for members, and communicating with mutual friends, among other activities.

The treatment process reduced family members' overprotectiveness of the PWUD, which fostered emotional bonding between the PWUD and other family members. The treatment's emphasis on mothers' and wives' attention to their personal demands, prioritizing their needs, and reducing their involvement with the PWUDs' needs are among other factors that enhanced the emotional closeness between the PWUD and disengaged family members (Panaghi et al., 2016). In most cases, disengaged family members were isolated due to the mother's or spouse's closeness to the PWUD. Thus, establishing clear boundaries between family members and the PWUD, and, therefore, eliminating unhealthy relationships, obviates the main reason for the disengagement of other family members (Wilkens & Foote, 2019).

These issues were related to the reorganization of boundaries between family members. Unlike the internal boundaries of families with PWUD, which are shattered and crushed (Saatcioglu et al., 2006), the boundaries around these families were rigid before the treatment. Rigid boundaries protect a family from the stigma associated with drug use (Lander et al., 2013). However, these boundaries became more flexible during treatment. That is, family members became less isolated. However, the relationship between family members and significant others was not close enough (Pilevari & Javadi, 2019) to let relatives and friends help to solve the family's problems. Families acknowledged that members outside the nuclear family sometimes have negative roles (Tsounis, 2013) and that receiving advice from them could make the treatment process more difficult.

Some of the main outcomes of treatment were the good feeling PWUDs felt when changing their lifestyle and the positive feedback they received from significant others; these outcomes enhanced PWUD's (and their families') desire to continue treatment. Since drug use strongly influences a PWUD's self-esteem (Oshri et al., 2017), people feel

worthless and helpless when on drugs. Their family and society consider them bad people, and people often ignore them. As a result, they resort to using drugs to achieve short-term comfort. Using drugs is an essential part of these people's lives. A PWUD may believe they cannot survive without drugs. They use the drugs as a protective tool against their self-esteem (Akhter, 2013). However, undergoing addiction treatment makes PWUD more likely to participate in healthy behaviors and, in turn, receive favorable feedback from their significant others. During treatment, families also learn to pay more attention to their own needs, focus less on the addicted person and his/her needs, and set goals for themselves and strive to achieve them. These changes promote interest, motivation, and enthusiasm among PWUD and their families (Beattie, 1992).

Furthermore, providing consistent training to families and the PWUDs increased their awareness of addiction, its effects on interpersonal relationships, and their responsibility to improve the situation. This awareness consequently boosted their mutual and supportive behaviors. Although raising awareness alone is not enough to change a behavior, it is helpful when it accompanies other vital therapeutic factors (Alhyas et al., 2015). The results of the current study also suggest that the more the educational content and mechanisms of the treatment are consistent with Iranian socio-cultural factors, the easier it is for PWUD and their families to accept and trust such methods.

Cultural sensitivity and positive therapeutic environments significantly affect general acceptance of and commitment to treatment (Woidneck et al., 2012). In Iranian culture, individuals are expected to control their true feelings and beliefs and speak according to what is socially acceptable (Beeman, 1986). Also, in Islam, sins are confessed before God, and Muslims are prohibited from confessing their sins to other people (Nuri & Husayn, 1987). Therefore, any therapeutic methods that require self-disclosure face cultural barriers. The PWUD and their families in this study benefited from C60's therapeutic approach. Unlike members of twelve-step programs, the PWUD in this study were not concerned with confessing their wrongdoings and sins to others.

Also, Iranians' strong tendency toward heroism (Sreberny & Mohammadi, 1994) can cause them to make heroes out of their therapists when they help them in difficult situations. Making heroes out of therapists plays a significant role in patients' trust in and acceptance of them. This is vital, as a patient's trust in a therapist is one of the main factors that promotes treatment continuation (Manju et al., 2016).

According to literature, heroes in Iran should commit to and act in line with the values that people have ascribed to them—otherwise, they will lose their followers. These issues are closely linked to the morals accepted in Iranian culture, such as magnanimity, honesty, and forgiveness (Ghaemi, 2011). In other words, as soon as people see behavioral or ethical flaws in their self-made heroes (therapists), they become frustrated and relinquish their commitment to them and their teachings. At C60, the hero is not close to his/her followers (clients and their families), and the responsibility of treating clients and their families is accepted by those who have recovered.

Individuals seeking treatment, and their families, are often desperate and confused, and they have typically undergone many unsuccessful treatments. They do not know where to go for treatment and, as a result, become vulnerable to immoral service providers. C60's unconditional acceptance of clients and refusal to receive money for the treatment convinced PWUD and their family members in this study that their therapists were not seeking economic benefits. This, in turn, made the PWUD believe that the therapists were genuinely willing to help them. The unconditional acceptance of clients also gave PWUD reason to accept and trust their therapists. Additionally, the director of C60 often had his wife and daughter at C60, which reassured the PWUD that the environment was safe for their families. According to the PWUD in our study, if sexual or emotional abuse of PWUD's family members was common, the director of C60 would not want his family in the treatment setting. Incorporating cultural components—trust, acceptance, and positive interpersonal attachment—increased the likelihood that PWUDs would be successful in treatment (Darchuk, 2007; Fletcher et al., 2015).

Based on this study's findings, peer group attendance and the design of the treatment process are two critical components of successful treatment outcomes. Attending peer groups made families feel less lonely. Listening to the stories and problems of other group members gave families and PWUD hope that the treatment would work and reduced their suffering. Moreover, the peer group allowed families to interact with others and free themselves from feeling isolated. At the same time, interacting with group members created new friendship networks and even rekindled past friendship networks (with a new function and far from drug use) for the PWUD. The results of various research studies indicate that group therapy increases the commitment of clients (Tracy & Wallace, 2016) and reduces the feeling of loneliness (CSAT, 2005) and isolation in them (Lander et al., 2013).

In addition to group therapy participation, easy access to treatment services and the therapist's use of understandable narrative language (often accompanied by folklore stories and proverbs) were also considered effective strategies for obtaining desirable treatment outcomes. The dispersion of branches of C60 across the city has, to a large extent, helped to bring together families and individuals with similar economic, social, and cultural backgrounds. Families with higher economic, social, and cultural status, whose PWUD relative had previous experience in mid-term residential treatment centers, were delighted with this feature because they believed that PWUD who belonged to a lower economic status had likely experienced worse situations, which could negatively impact the their family's PWUD. Mid-term residential treatment centers are often located outside the city and accommodate people belonging to various economic, social, and cultural statuses.

Many studies have investigated the importance of easy access to treatment services (Dengo-Baloi et al., 2020; Marsh et al., 2000), the provision of transportation services, and flexible scheduling (Owens et al., 2018; Rapp et al., 2006), as well as the use of simple language that is familiar to the client (Yu et al., 2009). According to the current findings, properly designing the treatment processes is crucial to encouraging PWUD and their families to engage in treatment.

The current study has several limitations. First, this study did not involve women who use drugs. Women with addiction in Iran face a particular taboo, and the economic, social, and cultural conditions that govern women's status make their addiction treatment difficult in Iran. Second, this study selected participants from among C60 clients. Hence, the results should be considered for other groups with caution until research examines them in a larger sample. Furthermore, since the researchers were not allowed to conduct interviews with newcomers to C60, we could not compare the changes in families' patterns at the time of their arrival at C60 to the start of the study. As such, we accepted participants' statements about their past and the changes that had taken place in their families since their treatment began.

5. Conclusion

Peace among family members promotes change among PWUD. Changing families' behavioral patterns can create peace within a family. However, changing existing patterns requires therapists for the family members, as well as for the PUWD.

The therapist serves as the glue that joins the family and the PWUD. According to Iranian culture, the presence of a spouse, daughter, mother, sister, or other woman, along with the therapists, during treatment makes the treatment environment feel more secure for the PWUD. This feeling of security, combined with C60's insistence on receiving little (or no) money in exchange for treatment services, increased PWUDs' and their families' trust in therapists in this study.

Treatment that incorporates cultural components, the availability of various training sessions based on simple and understandable methods, and easy access to drug addiction treatment centers are among the most

significant treatment conditions that influence the families of PWUD during treatment.

CRediT authorship contribution statement

All persons who meet authorship criteria are listed as authors, and all authors certify that they have participated sufficiently in the work to take public responsibility for the content, including participation in the concept, design, analysis, writing, or revision of the manuscript. Furthermore, each author certifies that this material or similar material has not been and will not be submitted to or published in any other publication before its appearance in the Journal of Substance Abuse Treatment.

Activity title	Azam Pilevari	Mohamad Zahedi Asl
Idea	✓	✓
Designing the project	✓	✓
Reviewing the literature	✓	
Designing interview questions	✓	✓
Selecting participants for the interviews and coordinating with them	✓	✓
Data collection, transcript audio into text	✓	
supervising the project		✓
Data analysis	✓	
writing the manuscript	✓	
Manuscript editing	✓	
Manuscript review	✓	✓

Declaration of competing interest

In the present study, the authors have no conflict of interest.

Ethical approval for this project was given by the Graduate Studies Council of the Faculty of Social Sciences, Allameh Tabatabaei University [Number 245].

Acknowledgment

The authors of this study would like to thank clients, families, and service providers of addiction treatment in C60.

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